Chronic Kidney Disease (CKD): understanding your kidney reports and care

> *A practical guide to understanding kidney reports, blood pressure and anaemia in chronic kidney disease. It explains the main points in everyday language. Your own treatment plan should always be discussed with your clinician.*

1. What does CKD mean?

Chronic kidney disease means that a problem with kidney structure or function has lasted for at least 3 months. One high creatinine result or one low eGFR result does not automatically mean CKD. Dehydration, an illness or some medicines can temporarily affect kidney test results, so your clinician may repeat the tests.

2. Three things help explain your kidney reports

Your clinician looks at what may have caused the problem, how well the kidneys are filtering (eGFR), and whether protein is leaking into the urine (urine ACR). These three parts are called CGA classification. Even when eGFR looks fairly good, protein in the urine can be a sign of kidney disease.

3. GFR and albuminuria categories

G1 ≥90; G2 60–89; G3a 45–59; G3b 30–44; G4 15–29; G5 <15 mL/min/1.73 m^2. Urine ACR: A1 <30 mg/g; A2 30–300 mg/g; A3 >300 mg/g. Prefer a spot urine ACR (often first-morning); confirm abnormality in context. Risk rises as eGFR falls and albuminuria increases.

4. Make sure blood pressure is measured properly

Sit quietly for a few minutes before the reading. Use the right cuff size, keep your back supported and feet on the floor. The KDIGO target of a top BP number below 120 is meant for readings taken using a specific standardized method—not every routine reading. Home BP records can also help your clinician understand your usual BP.

5. What blood pressure should you aim for?

For many adults with CKD who are not on dialysis, KDIGO 2021 suggests a top BP number below 120 mmHg, if it is measured in the standardized way and the person feels well with treatment. This may not suit someone who gets dizzy on standing or has other serious health problems. If urine protein is present, your clinician may prescribe an ACE inhibitor or ARB. Do not start or stop these medicines yourself.

6. What should be checked after starting an ACE inhibitor or ARB?

BP, creatinine and potassium are usually checked 2–4 weeks after starting one of these medicines or increasing its dose. If creatinine rises by more than 30%, the reason needs to be checked. If potassium rises or BP becomes too low, your clinician may adjust treatment. Do not stop the medicine just because one report has changed—contact your clinician.

7. Salt, food and staying active

For people with high BP and CKD, keeping salt below about 5 grams a day may help. This advice is not right for everyone, so do not make major diet changes without advice. Some low-sodium salts contain potassium and can be unsafe in kidney disease. Keep moving in a way that suits your strength and general health. If possible, aim for about 150 minutes of moderate activity each week.

8. Anaemia (low blood count) in CKD

Some people with CKD develop anaemia. Tell your clinician if you feel unusually tired, weak or breathless. Tests may include a full blood count (CBC), reticulocyte count, ferritin and TSAT. KDIGO 2026 suggests checking at least once a year in G3, twice a year in G4, and every 3 months in G5 or dialysis. You may need testing sooner if symptoms or bleeding occur.

9. Iron treatment thresholds — haemodialysis

In anaemic CKD G5 haemodialysis, consider iron when ferritin is at or below 500 ng/mL and TSAT is at or below 30% (2D). IV iron is suggested over oral iron when initiating therapy.

10. If you are not on haemodialysis, or you are on peritoneal dialysis

For people with CKD who are not receiving haemodialysis, including those on peritoneal dialysis, iron may be considered when ferritin is below 100 ng/mL with TSAT below 40%, or when ferritin is 100 to below 300 ng/mL with TSAT below 25%. The decision to use tablets or intravenous iron depends on symptoms, severity, tolerance, likely response, availability and cost. Do not start iron solely from one number without reviewing the full blood count and clinical context.

11. When should routine iron be withheld?

During routine iron treatment, consider withholding iron when ferritin is above 700 ng/mL or TSAT is 40% or higher. These are treatment-monitoring thresholds, not targets to chase. Your clinician will also consider infection, inflammation, recent iron doses, haemoglobin trend and the reason for anaemia. Do not start, stop or change iron treatment yourself.

12. Monitoring and other anaemia treatments

After starting iron, haemoglobin, ferritin and TSAT need repeat monitoring. If oral iron is not tolerated or does not work well enough after an appropriate trial, your clinician may consider intravenous iron. Before an ESA or HIF-prolyl hydroxylase inhibitor is considered, look for treatable causes such as iron deficiency, blood loss, infection or inflammation, and vitamin B12 or folate deficiency. ESA treatment is generally preferred over HIF-PHI after correctable causes are addressed. For adults receiving an ESA, the haemoglobin target should remain below 11.5 g/dL. Transfusion decisions depend on symptoms, urgency and clinical context—not a single haemoglobin number alone.

> ### १३. रोजच्या उपचारात काळजी घ्या
>
> किडनीवर परिणाम करू शकणारी painkiller औषधे (विशेषतः NSAIDs) डॉक्टरांचा सल्ला न घेता घेऊ नका. काही औषधांची मात्रा किडनीच्या कामानुसार बदलावी लागते. BP, creatinine/eGFR, potassium आणि लघवीतील प्रथिने यांचे रिपोर्ट वेळोवेळी पाहा. लघवी अचानक खूप कमी होणे, सूज किंवा दम झपाट्याने वाढणे, छातीत दुखणे, बेशुद्ध पडणे किंवा potassium खूप वाढल्याचा संशय असल्यास तातडीने वैद्यकीय मदत घ्या.
>
> For a life-threatening emergency in India, call 112. Elsewhere use your local emergency number.

१४. पुढच्या भेटीपूर्वी ही यादी पाहा

☐ किडनीचा त्रास ३ महिन्यांपेक्षा जास्त काळ आहे का हे स्पष्ट आहे? ☐ eGFR आणि urine ACR चे रिपोर्ट आहेत का? ☐ BP योग्य पद्धतीने मोजला आहे का? ☐ औषधे आणि त्यांची मात्रा डॉक्टरांनी तपासली आहे का? ☐ Hb, ferritin आणि TSAT तपासण्यांची तारीख ठरली आहे का? ☐ किडनी तज्ज्ञांचा सल्ला कधी घ्यायचा हे ठरले आहे का?

Dialysis and everyday kidney care — What the urine does not tell you

Some people retain kidney function after dialysis begins. Passing urine does not by itself show that waste, potassium and fluid are adequately controlled. Dialysis removes waste and excess fluid; it is not a cure for kidney failure.

Dialysis and everyday kidney care — How your plan is chosen

Your team considers blood results, symptoms, fluid balance, remaining kidney function and treatment adequacy. **Keep the prescribed schedule.** Discuss travel, work and financial difficulties rather than skipping a session. **Do not reduce frequency** because another person has fewer sessions.

Dialysis and everyday kidney care — Food and fluid need their own plan

There is no universal one-litre allowance or very-low-protein diet for everyone on dialysis. Nutrition needs differ with treatment and health. Ask how drinks and liquid foods count towards an allowance, and bring your medicine list and recent results.

Dialysis and everyday kidney care — A useful home record

Record the observations requested by your team: weight, swelling, breathing, appetite and changes in urine. Ask which changes need a same-day call and who to contact after hours. This record supports, rather than replaces, laboratory monitoring.

> ## Dialysis and everyday kidney care — Seek help promptly
>
> Contact the dialysis team the same day after a missed session or marked new swelling. Severe breathlessness, chest pain or collapse requires emergency care, not an online appointment.
>
> For a life-threatening emergency in India, call 112. Elsewhere use your local emergency number.

Dialysis and everyday kidney care — Potassium has a useful job

Potassium supports nerves, muscles and the heart. Healthy kidneys remove excess mainly in urine. Kidney disease can reduce removal; it does not mean that no potassium ever leaves every patient’s body.

Dialysis and everyday kidney care — Kidney disease does not always mean high potassium

Levels may be high, normal or low. Blood results, kidney function, dialysis, medicines, supplements and potassium-containing salt substitutes all matter. **Do not stop prescribed medicines on your own.**

Dialysis and everyday kidney care — Where coconut water fits

Coconut water contains potassium. If your potassium is high, you have been told to restrict it, or you have a fluid limit, avoid it until your team confirms what is appropriate. A permanent ban for every healthy person or every kidney patient is not a universal rule. It is not a kidney-cleansing treatment.

Dialysis and everyday kidney care — Leaching has limits

Preparation can reduce some potassium in selected vegetables; **it does not remove all of it**. Ask a renal dietitian which foods, methods and portions suit you. Do not assume a leached vegetable can be eaten without limit or apply the method to every vegetable without need.

Dialysis and everyday kidney care — Ask three practical questions

What is my latest potassium result? Which portions are suitable? When should it be checked again? Urine volume and feeling well cannot replace a blood test. Seek a workable meal plan rather than removing all fruit and vegetables.

> ### Dialysis and everyday kidney care — Do not treat dangerous results at home
>
> Follow urgent instructions for a dangerous potassium result even without symptoms. Palpitations, severe weakness, collapse or chest symptoms needs urgent assessment. Avoiding a drink or boiling vegetables is not emergency treatment.
>
> For a life-threatening emergency in India, call 112. Elsewhere use your local emergency number.

Questions you may have

My eGFR is 58 on one report. Does that mean I have CKD?

Not necessarily. Your clinician will look at earlier and repeat results, urine ACR and other signs of kidney damage before confirming CKD.

Does everyone with CKD need a BP below 120?

No. This target is for adults not on dialysis and applies when BP is measured in a standardized way and the person can tolerate it. Your clinician may choose a different target.

Should I stop ACEi or ARB if creatinine rises?

Do not stop it on your own. A blood test is usually checked 2–4 weeks after starting or increasing the dose. If creatinine rises by more than 30%, your clinician will look for the reason and decide what to do.

Does the KDIGO 2026 anaemia guideline cover all CKD treatment?

No. It focuses on checking and treating anaemia in people with CKD.

Can I reduce sessions if I still urinate?

Only the nephrology team can change your prescription after assessing the whole picture.

Does leaching remove all potassium?

No. Some remains, so portions and the overall plan still matter.

Does passing urine prove potassium is normal?

No. Your blood result and clinical assessment are needed.

What to remember

Use both eGFR and urine ACR to understand CKD. Set blood pressure and treatment goals according to the person, not just one report. Keep follow-up tests on time and never change medicines on your own.

Sources and further reading

KDIGO 2024 — CKD Evaluation and Management guideline
https://kdigo.org/guidelines/ckd-evaluation-and-management/

KDIGO 2021 — Blood Pressure in CKD guideline
https://kdigo.org/guidelines/blood-pressure-in-ckd/

KDIGO 2026 — Anemia in CKD guideline
https://kdigo.org/guidelines/anemia-in-ckd/

NIDDK — hemodialysis
https://www.niddk.nih.gov/health-information/kidney-disease/kidney-failure/hemodialysis

NIDDK — healthy eating with chronic kidney disease
https://www.niddk.nih.gov/health-information/kidney-disease/chronic-kidney-disease-ckd/healthy-eating-adults-chronic-kidney-disease

NIDDK — eating and nutrition on hemodialysis
https://www.niddk.nih.gov/health-information/kidney-disease/kidney-failure/hemodialysis/eating-nutrition

American Heart Association — changes to report
https://www.heart.org/en/health-topics/heart-failure/living-with-heart-failure-and-managing-advanced-hf/physical-changes-to-report-for-heart-failure

NIH Office of Dietary Supplements — potassium
https://ods.od.nih.gov/factsheets/potassium-Consumer/

National Kidney Foundation — potassium and food preparation
https://www.kidney.org/kidney-topics/potassium-your-ckd-diet

National Kidney Foundation of Hawaii — coconut water
https://kidneyhawaii.org/blog/is-coconut-water-good-for-my-kidneys/

National Kidney Foundation — high potassium and urgent treatment
https://www.kidney.org/kidney-topics/hyperkalemia-high-potassium

Government of India — Emergency Response Support System 112
https://112.gov.in/about

Educational information supports understanding and does not replace individual medical care. Follow your clinician’s plan. Do not change medicines on your own. Sources checked: 24 October 2026.

Website: https://home-visiting-icu-doctor.vercel.app/#wikiSection
Article: ckd-kdigo | en